

What AI changes in the consultation, and what you bring to it

A one-page reminder of the ATOC session at the BOA Annual Congress on 24 September 2026: the three ideas the talk depends on, three things to do tomorrow, and where to read more.

TERM Context sovereignty Controlling your own professional context (your specialism, your patients, local pathways, how you reason) and using it to shape what AI does for you, whichever provider you use. AI systems don't remember you between sessions and don't know your field or your patients, so that context is what makes their output useful. Your patients hold the same claim over theirs.		TERM Sanctuary strategies Four common responses to AI in practice: denial ("it can't replicate the therapeutic relationship"), retreat to what only humans can do, restriction, and resignation. Each of them leaves the clinician watching something happen to them rather than shaping it.		TERM Agent An AI system that keeps context across time, plans a sequence of steps, and acts: it retrieves records, drafts notes, and books follow-ups. It moves AI from a tool you use to a system acting on your behalf, and your patients will increasingly bring their own into the consultation.	
01 Write down your professional context Your specialisation, patient population, CPD priorities, and how you reason, on one page and explicit enough to give to an AI. Paste it in before you ask for anything clinical, and add to it as your practice changes. Keep patient-identifiable information out of it.		02 Give one patient a structured prompt "Before your next appointment, ask your AI what questions you should be asking about your progress with your condition, and what you should understand about the next phase of treatment." Patients are already using AI before they arrive; this helps them use it well and gives you a better conversation.		03 Bring judgement, not deference Read the next AI output you meet as you'd read a colleague's opinion: engaged, contextual, and accountable. AI makes different errors from the ones people make, so the checking stays with you, and so does the synthesis.	
SESSION Making sense of AI in clinical practice		HOST ATOC session, BOA Annual Congress, London		DATE 24 September 2026	
				AUDIENCE Trauma and orthopaedic physiotherapists	

FURTHER READING

The slides, references, and a longer account of the talk: michael-rowe.github.io/home-michael/Presentations/2026-09-24-atocp-making-sense-of-ai.

Context sovereignty for AI-supported learning: the longer argument behind the central idea of the talk

Context engineering for educators: the practical side of giving AI the context it needs, written for educators but just as true in the clinic

How to replace a physiotherapist: Rowe, Nicholls and Shaw (2022) on AI and the redistribution of work in healthcare

The evidence in the talk comes from studies published between 2023 and 2026, and it's moving quickly; the full reference list is at the end of the slides. If you try any of the three things above, I'd like to hear how it went: mrowe@lincoln.ac.uk.